A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

17 April 2026

Dr Dejan Fiser
Medical One

Dear Dr Fiser,

Re: Mr Bryn Murrell
DOB: 1997-07-25
Address: 507/11 Rose Lane, MELBOURNE, VIC, 3000

Summary

- **Significant improvement in irritable bowel syndrome symptoms on current management.**
- **Symptom control maintained with fibre supplementation and a 37.5 mg dose of Nortryptyline**
- **Plan for maintenance of current therapy for a further eight to ten months prior to a trial of cessation.**
- **No action required by you.**

I had the pleasure of reviewing Bryn in the clinic to evaluate his progress regarding his gastrointestinal symptoms. You will be pleased to note that he reports a substantial improvement since his initial presentation nearly a year ago. At that time, he was experiencing daily cramping, persistent loose stools, and significant tenesmus.

Bryn's symptoms are now well-controlled. He reports that his bowel movements have largely normalised in frequency and consistency, which he attributes to the combination of his current medication and diligent fibre supplementation with Metamucil. He noted that the cramping has become a rare occurrence, only resurfacing briefly during a recent lapse in medication adherence due to a pharmacy dispensing error.

We discussed the influence of physiological stressors on his condition. Bryn observed that an acute stressful event recently triggered a transient episode of softer stools and increased urgency, confirming a continuing, though much improved, stress-sensitive component to his bowel habits. He

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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is currently tolerating the 37.5 mg dose of his medication well and denies any significant side effects such as xerostomia or somnolence.

Impression and Plan Bryn's irritable bowel syndrome is currently in a period of stable remission. My clinical preference is to maintain symptom stability for a total of nine to twelve months before attempting to wean pharmacotherapy. As he has achieved excellent control over the past two months, I have advised him to continue his current 37.5 mg dose for an additional eight to ten months.

I have provided Bryn with the necessary prescriptions to continue this programme. We have agreed that a further formal specialist review is not required at this time. He will follow up with you for his routine care, and we have discussed a tentative plan to trial a slow cessation of his medication towards the end of the year. I have counselled him that should his symptoms recur upon cessation, he may either resume the medication under your guidance or return for a further consultation.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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